

Suicide—safer care, built into the **EHR**

Built around people. Enabled by technology. A Zero Suicide—aligned initiative at no cost to participating sites.

SPIER embeds evidence-based suicide screening, assessment, and safety-planning tools **natively into electronic health records** — written as discrete, coded data instead of forms attached to the chart — so risk is visible, triggers the right next step, and follows the patient across care settings.

WHY NATIVE, CODED TOOLS MATTER

Many health systems have rebuilt screening tools in EHR form builders — but the result is usually a document attached to the chart. Nothing downstream can act on it, and no receiving clinician has time to open ninety CCDs looking for suicidality.

When a PHQ-9 or C-SSRS writes to a **coded observation**, the EHR can flag risk, queue the follow-up assessment, and share a machine-readable signal with the next facility — even one on a different EHR.

THE APPROACH

1 Capture

Standardized tools built into the EHR base product, writing discrete LOINC/SNOMED-coded results — deployed vendor-wide, not site by site.

2 Translate

Tool-specific results map to a common suicide-risk indicator, so sites using different instruments can still understand each other.

3 Act

Coded results trigger follow-up workflows and recommendations — within a site, and across sites through health information exchange.

WHAT SPIER PROVIDES — AT NO COST

- ✓ **Tool selection guidance** matched to your settings and workflows (ED, inpatient, primary care, behavioral health)
- ✓ **Complete developer build specs** — FHIR artifacts plus visual build references for every tool
- ✓ **Adoption guide and data dictionary** with LOINC and SNOMED codes for each instrument and answer
- ✓ **Licensing agreements** negotiated centrally with tool authors
- ✓ **Clinician training and implementation support**, grounded in the Zero Suicide framework
- ✓ **Research and evaluation partnership** to measure impact — including statewide, cross-site outcomes

LICENSING, ANSWERED

Most instruments carry **no cost to sites**. SPIER holds or arranges the agreements, and helps match tools to the right setting — including which tools are appropriate for patient self-completion and which require clinician administration.

PUBLIC DOMAIN	ASQ (NIMH), PHQ-9, SBQ-R, PSS-3, BSSA
FREE W/ REGISTRATION	Columbia C-SSRS (screener and full)
LICENSED + TRAINING	Stanley-Brown Safety Plan, CAMS — agreements in place; authors require clinician training before use
VENDOR-LEVEL	EHR-vendor licensing (e.g., Oracle Health licensing for its whole client base) is negotiated separately from site licensing — SPIER supports both paths

The SPiER suicide-safer care pathway

Eight EHR-supported stages, aligned to the Zero Suicide framework — each with validated tools that write structured, shareable data.

Tool FHIR specification available today

Tool Catalogued — on the roadmap

1 Identify Possible Risk

Find a suicide-risk signal at any point of care and determine whether more review is needed.

ASQ

PHQ-9 Item 9

C-SSRS Screener

SBQ-R

C-SSRS Pediatric

PSS-3

Positive-Screen Workflow Trigger

2 Clarify Risk

Determine what is going on clinically: thoughts, plan, intent, behavior history, access to means, risk and protective factors.

C-SSRS Full

CAMS SSF-5

NIMH BSSA

C-SSRS Since Last Contact

CARS-S

Local Assessment



Automated hand-off between stages: a positive ASQ result or any endorsement of PHQ-9 Item 9 automatically surfaces the Clarify Risk assessments — no one has to remember to order the next step.

3 Define the Risk Picture

Document the current risk status and the clinical reasoning that guides next steps.

CAMS Therapeutic Worksheet

SAFE-T

4 Document Safety Actions

Record the concrete actions taken to reduce risk — safety planning, lethal-means safety, crisis resources.

Stanley-Brown Safety Plan

CAMS Stabilization Plan

Lethal Means Safety / CALM

Crisis Response Plan

Crisis Resources (988)

5 Coordinate Handoffs

Transfer safety information, responsibility, and follow-up details across people, settings, and time.

Safety Handoff Checklist

Discharge Safety Packet

Referral Handoff

Next-Appointment Scheduling

Consent / Sharing Status

6 Track Follow-Up

Track whether outreach and follow-up actually happen after the immediate encounter.

Outreach / Contact Attempts

Caring Contacts

Appointment Tracking

No-Show Follow-Up

Escalation Workflow

7 Track Risk Over Time

Keep active suicide-safer care episodes visible, trackable, and escalated when needed.

Registry / Work Queue

Episode Status

Reassessment Schedule

Care-Gap Tracking

Overdue Escalation

8 Measure & Share the Data

Make pathway activity usable for quality improvement, reporting, and cross-site information sharing.

KPI / Measure Reporting

Dashboard

Analytics Extract

Interoperability Output

DISCRETE & CODED

Every tool writes **LOINC/SNOMED-coded observations**, not attached documents — data the EHR, analytics, and AI tools can actually use.

ONE COMMON RISK SIGNAL

Tool-specific results roll up to a **shared suicide-risk indicator**, so a patient screened with the ASQ at one site is recognized at a site using the C-SSRS.

ACROSS SETTINGS & EHRs

Structured results travel through **health information exchange**, flagging risk when patients move between facilities and platforms.